

**Statement of Work
Wagner Indian Health Service
Mid-Level Providers for Outpatient Department**

Background

The Indian Health Service (IHS) an agency within the Department of Health & Human Services is responsible for providing federal health services to American Indians and Alaska Natives. The Indian Health Service provides a comprehensive health service delivery system for approximately 1.9 million American Indians and Alaska Natives who belong to 567 federally recognized tribes, in 35 states. The Indian Health Service Headquarters is located in Rockville, MD and then divided into twelve physical areas: Alaska, Albuquerque, Bemidji, Billings, California, Great Plains, Nashville, Navajo, Oklahoma, Phoenix, Portland and Tucson.

The Great Plains Area Office (GPAO) works in conjunction with its 19 Indian Health Service Units and Tribal Managed Service Units to provide health care to approximately 122,000 Native American located in North Dakota, South Dakota, Nebraska, and Iowa. The Area Office's service units include seven hospitals, eight health centers, and several smaller health stations and satellite clinics.

The Wagner Indian Health Center is an ambulatory clinic with extended clinic hours, which falls under the GPAO. The clinic hours are from 7:00 a.m. to 11:00 p.m. daily, from Sunday to Saturday, including Federal holidays. Wagner Indian Health Center provides medical care for approximately 23,221 tribal members. Serving a rural area, many of our patients travel 100 miles' round trip to receive care. Services include Pharmacy, Public Health Nursing, Radiology Services, Outpatient Care, Laboratory, Dental, Community Health, Purchase Referred Care, Dietitian, Optometry, Pain Management, Behavioral (on site) and Specialist Clinics: OB/GYN and Telemedicine: Cardiology, Nephrology and Behavioral.

Objective

To provide uninterrupted Outpatient Service for the Wagner Indian Health Center.

Period of Performance

Date of award till one year from date of award.

Tour of Duty/Work Schedule

Mid-level Provider Outpatient Clinic:

Working Sunday to Saturday – 7:00 a.m. to 11:00 p.m.

Place of Performance

Wagner IHS Healthcare Center
111 Washington Ave., NW
Wagner, South Dakota 57380

DESCRIPTION OF SERVICES:

The contractor shall provide a minimum of three (3) Outpatient Mid-Level Provider(s) (i.e. Certified Nurse Practitioner or Certified Physician Assistant) for Outpatient Services at the Wagner Indian Health Center. There will be only one (1) provider covering the tour of duty/work schedule.

The mission of the Wagner Indian Health Center in Wagner, South Dakota is to provide the best possible health care services to the members of the Yankton Sioux Tribal Members (Ihanktonwan Nation) in South Dakota.

The Great Plains Area Indian Health Service (GPAIHS) requires contractor support to assist in the recruitment and placement of an Outpatient Midlevel Provider into temporary duty assignments in health centers and hospitals in the Great Plains Area IHS. The purpose of the contract is for the recruitment, credentialing, and placement of an Outpatient Midlevel Provider at Wagner Indian Health Center.

This contract is a non-personal health care services contract, as defined at Far Part 37.101 under which the Contractor is an independent contractor and which shall allow the Government to evaluate the quality of professional and administrative services provided, but retains no control over the medical, professional aspects of the services rendered, (e.g., professional judgements, diagnosis for specific medical treatment). The Contractor shall indemnify the Government for any liability producing act or omission by the contractor, its employees and agents occurring during contract performance. The Contractor shall maintain medical liability insurance, in the coverage amount acceptable to the Contracting Officer, which is not less than the amount normally prevailing within the local community for the medical specialty concerned.

Upon the receipt of an IHS purchase order, the Contractor shall locate, credential, and have available sufficient staffing to provide an Outpatient Midlevel Provider coverage for the Outpatient Department Sunday to Saturday 7:00 a.m. to 11:00 p.m. Regular workweek, weekend, shift coverage and block scheduling hours are available time options.

The standards of medical practice and duties of the Contractor through its providers shall be determined pursuant to Governing Body bylaws, medial staff bylaws, rules and regulations of the appropriated IHS clinic, applicable provisions of law including but not limited to rules and regulations of any and all governmental authorities relating to licensure and regulation of physicians and hospitals, and standards and recommendations of CMS (Centers for Medicare and Medicaid Services) compliance. The Contract Provider(s) shall be required to participate in a quality improvement program, which is consistent with current requirements for ongoing monitoring and evaluation of the quality and appropriateness of care.

The contract resulting from this solicitation shall:

1. be a non-personal health care services contract under which the Contractor is an independent contractor.
2. allow the Government to evaluate the quality of professional aspects of the services rendered, (e.g., professional judgments, diagnosis for specific medical treatment);
3. require the Contractor to indemnify the government for any liability-producing act or omission by the Contractor, its employees and agents occurring during contract performance.

4. require the Contractor to maintain medical liability insurance, in the coverage amount acceptable to the contracting officer, which is not less than the amount normally prevailing within the local community for the specific specialty; and
5. require the Contractor to ensure its subcontracts for provisions of health care service, contain the requirements of the clause at FAR 52.237-7, including the maintenance of medical liability insurance.

1.1 CONTRACTOR RESPONSIBILITY

- 1.1.1. Provides on-site Outpatient Midlevel medical services to beneficiary patients that present for care to the Wagner IHS Outpatient Department (OPD). The Outpatient Department sees approximately 7,500 patient visits per month ranging in age from newborn to geriatric. Due to the volume of patients, Outpatient Midlevel Provider must be able to provide high quality, effective and efficient care to patients. There is no pre-set limit of the patients per day that the provider shall see but on average it could be 10 to 16 per day.
- 1.1.2. The incumbent must possess specialized knowledge of the concepts, principles and practices of Internal Medicine or General Medicine, Pediatrics or Family Practice, or Obstetrics and Gynecology such as is obtained through the successful completion of a course of study leading to the Accredited Nurse Practitioner or Physician Assistant Program and the successful completion of post graduate training. Completion of Board Certification.
- 1.1.3. The Outpatient Midlevel Provider's actual clinical duties shall be in accordance with his/her credential privileges.
- 1.1.4. The Outpatient Midlevel Provider shall practice within the Medical Staff Bylaws of the Wagner Indian Health Center.
- 1.1.5. Primary duty shall be to provide outpatient care in OPD. Examines and diagnoses the health-related conditions of individuals requesting care, refers individuals for consultation when appropriate, prescribes and carries out, or directs others in carrying out, appropriate therapy in conformance with approved Clinical Privileges. Educates individuals in the nature of health-related conditions, recommended therapy and in the general promotion of health and prevention of disease. Documents consultations on the patient's health care record.
- 1.1.6. Implements, evaluations and are consulted on Outpatient procedures and instructions in accordance with their clinical privileges.
- 1.1.7. Establishes inter-personal and intra-departmental relationships to enhance optimal patient care.
- 1.1.8. Demonstrates competent and effective oral and written communication.
- 1.1.9. Utilizes RPMS/EHR computer software/program for entering and signing medical orders such as laboratory studies, and directly signing and dating radiological studies, etc.

- 1.1.10. Obtains necessary information on patients seen in the Outpatient Department, such as medical records, x-ray films and reports, and pathology slides and reports and documents pertinent normal and abnormal test results in the patient's medical record.
- 1.1.11. Maintains familiarity and complies with clinical antiseptic policies and procedures. Maintain familiarity and comply with all Federal, State, Indian Health Service, CMS regulations, and departmental regulation and requirements.
- 1.1.12. Ensure a safe work environment and employee safe work habits.
- 1.1.13. **REFERRALS AND CONSULTS.** The provider shall follow IHS Purchased Referred Care regulations and policies when arranging for a referral or consultation. The Outpatient Midlevel Provider shall be required to consult with other specialty practitioners for consultative opinions and continuation of care. The provider shall inform patients of the required referral or consultation by indicating the specialty involved.
- 1.1.14. **PRESCRIPTIONS.** The provider shall use and follow the IHS formulary for prescriptions for all patients. The formulary list is approved and maintained by the Wagner IHS Pharmacy and Therapeutics Committee. Any drug not listed on the formulary must be approved by the Pharmacy and Therapeutics Committee.
- 1.1.15. **CONTRACTOR TIMESHEETS.** The Outpatient Midlevel Provider shall be responsible for obtaining Clinical Director or representative signature for the completion of their timesheet. Invoices submitted without the signed timesheet Clinical Director's signatures shall not be paid.
- 1.2 PROCEDURE GUIDANCE.** Contractor providers shall perform procedures compatible with the medical facility's operating capacity and equipment. New medical procedures/services shall not be introduced without prior recommendation to, and approval of the Clinical Director or authorized representative.
- 1.3 PRIVACY OF INDIVIDUALLY IDENTIFIABLE HEALTH INFORMATION.** The Contract health care providers shall provide patients with the utmost care and attention. All patients shall be assured of their privacy and personal dignity. The Contractor shall ensure strict compliance to the release of Protected Health Information in accordance with the Code of Federal Regulation, Title 45, Public Welfare, Subtitle A, Department of Health and Human Services, Part 164, Security and Privacy (45 CFR 164) and AFI 41-210, "Patient Administration Functions" paragraph 2.2.3, "Health Insurance Portability and Accountability Act (HIPAA)" and paragraph 2.3, "General Guidelines on Releasing Medical Information".
- 1.4 COMMUNICATIONS.** The Contractor shall ensure contract providers maintain open and professional communication with members of the Medical Staff. Complaints validated by the Clinical Director, Chief Executive Officer, and Contracting Officer Representative shall be reported in writing to the Contracting Officer (CO) and the Contractor for action.
- 1.4.1 **ENGLISH LANGUAGE REQUIREMENT.** The Contractor provider(s) shall read, understand, speak and write English fluently.
- 1.5 DOCUMENTATIONS.** Contractor personnel shall prepare all documentation to meet or exceed established CMS standards and Wagner IHS Medical Staff Bylaws.
- 1.6 AVAILABILITY:**

HOURS OF OPERATION: Contractor coverage shall be for the Outpatient Department.
Tour of Duty: Sunday to Saturday – 7:00 a.m. to 11:00 p.m.

- 1.6.1 CONTINUITY OF SERVICES/REPLACEMENT STAFF. The Contractor point of contact shall ensure qualified personnel are available to provide coverage at all times even during scheduled absences of contract providers.
- 1.6.2 The Outpatient Midlevel Providers services are considered vital to the Government and must be continued with the minimal interruption. In order to minimize interrupted services, the incumbent contractor shall exercise its best efforts and cooperation to affect an orderly and efficient transition to a successor.
- 1.6.3 PERFORMANCE OF SERVICES DURING CRISIS DECLARED BY THE NATIONAL COMMAND AUTHORITY. On occasion, services may be required to support an activation or exercise of contingency plan outside the normal duty hours. Emergency situations (i.e. accident and rescue operations, civil disturbances, natural disasters) may necessitate the contractor provide increased or reduced with the contractor for the cost of these emergency requirements.

1.7 ORIENTATION. The Contractor shall ensure that all new contract providers participate in the Wagner IHS Facility Orientation with an overlap of services to include one additional day. Providers must arrive on Monday-Friday excluding holiday and participate in the orientation, **which must be completed in the Monday-Friday timeframe**, prior to providing services at the clinic.

1.8 RECORDS. The contractor shall document all patient encounters legibly in the patient electronic medical record. All documentation shall be timed and dated. Verbal orders are to be used as infrequently as possible. In the event of a major disaster or code all verbal orders are to be signed off immediately following the code or disaster. All Medical Records are to be completed within 24 hours of the provider's scheduled shift. All documentation shall meet the current CMS guidelines.

1.8.1. Final payment will be withheld until all medical records are completed by the contract provider.

1.9 PERSONNEL REQUIREMENTS

1.9.1 CONTINUING MEDICAL EDUCATION (CME) / CERTIFIED EDUCATION UNITS (CEU) REQUIREMENTS. Maintain CME and review pertinent journals, books and publications. Health care providers registered or certified by the national/medical associations shall continue to meet the minimum standards for CME to remain current. CME hours shall be reported to the credentialing office for tracking by the Clinical Director.

1.9.2 LICENSURE/REGISTRATION. Non-personal services contract personnel providing care at the Wagner IHS facility must maintain a current active and unrestricted license in at least one of the 50 United States or US territories.

1.9.3 CERTIFICATION. Nurse Practitioners and Physician Assistants shall be Nationally Certified by their respective National Professional Board and State Licensed.

1.9.4 FORMAL EDUCATION. The provider must show proof of graduation from an accredited school of their profession.

1.9.5 EXPERIENCE. The Provider must have a minimum of 3 years of experience within Family Medicine/General Medicine within the past 36 months prior to provider beginning performance at Wagner IHS.

1.9.6 Maintain certification in Basic Life Support (BLS).

1.9.7 LIABILITY INSURANCE. The Contractor shall maintain current medical liability insurance, in the coverage amount acceptable to the Contracting Officer, which is not less than the amount normally prevailing within the local community for the medical specialty concerned.

1.9.8 Computer efficiency required. Preferred Electronic Health Record experience.

2. PERFORMANCE-BASED SERVICE DELIVERY SUMMARY. The contractor's performance shall be measured based on the following Performance Requirements. The Performance Requirements shall be measured against Government Performance Standards and monitored in accordance with the following Methods of Government Surveillance or what is known as a Quality Assurance Surveillance Plan (QASP) to determine compliance. Deductions shall be assessed against individual invoices when services are not performed or do not meet contract requirements. Deductions will be made by the category that was deficient.

OUTPATIENT MIDLEVEL PROVIDERS

Performance Task/Requirement	Indicator	Standard (AQL) Acceptable Quality Level	Surveillance (QA) Quality Assurance	Incentive
Completion of documentation of patient record	All patients records must be documented accurately, correctly, and completely within 24 hours of patient visit	100%	Random observation and HIM Report	5% deduction on weekly invoices. Deductions will increase by 1% in subsequent months.
Prompt and continuous coverage	Agency must provide a competent Mid-Level to ensure no gaps in coverage.	100%	Provider Schedule to be reviewed at end of POP.	2% incentive bonus for contract company.
Credentialing	Agency allows 3 businesses to submit required credentialing documentation	100%	Credential Report	5% deduction on weekly invoices. Deductions will increase by 1% in subsequent months.

2.1 Performance Evaluation. If the contractor fails to perform required services at the acceptable quality level, payment may be withheld until the contractor makes corrections. In the event that the contractor fails to perform any required services, the COR shall provide written notice of such to the Contracting Specialist and CCO and the contractor. The Contracting Office will issue the contractor a Contract Discrepancy Report (CDR). The contractor shall have seven (7) calendar days to provide the Contracting Office with their response to the CDR. Upon evaluation of the contractor's response to the CDR for tasks surveilled, the contractor's payment for the month in which the performance is question occurred will be calculated as outlined above.

2.2 Contractor Payment. If the contractor fails to perform required services at the acceptable quality level, payment may be withheld until contractor makes corrections.

Contracting Officer's Representative Designation

The Contracting Officer Representative (COR) designated for this Project is:

Dwight Janis
Wagner IHS Healthcare Center
111 Washington Ave., NW
Wagner, SD 57380
Office: 605-384-4809

The COR is responsible for:

- a. Monitoring the Contractor's technical progress.
- b. Interpreting the statement of work.
- c. Technical evaluation is required.
- d. Technical inspections and acceptance required by this contract; and,
- e. Assisting the Contractor in the resolution of technical problems encountered during performance of this contract.

Subject matter expert:

Dr. Antonio Padilla-Cedo, Clinical Director
Wagner Indian Health Center
111 Washington Ave., NW
Wagner, SD 57380
Office: 605-384-4834

2.3 Compensation. The contractor shall be paid a fixed flat rate per hour. No differential pay for government holidays, compensation time, and night differential. There is no overtime.

2.4 Quality Assurance Surveillance Plan (QASP). The purpose of QASP is to provide a government developed system to assure the use of quality assurance methods in the administration of the performance-based acquisition standards. The intent is to ensure that the Contractor performs services for the Wagner Indian Health Center that are provided in accordance with performance requirements set forth in the contract documents, that the government receives the quality of services called for in the contract and that the government only pays for the acceptable level of services received.

The plan provides the basis for the COR to evaluate the quality of the contractor's performance. The oversight provided for in the contract and in this plan is implemented to help ensure that service levels reach and maintain the required levels throughout the contract term. Further, this plan provides the COR with a proactive way to avoid unacceptable or deficient performance and

provides verifiable input for the required past performance evaluations.

3. GOVERNMENT FURNISHED PROPERTY AND SERVICES.

3.1 EQUIPMENT. Contract providers shall have joint use of all available equipment for performing services required by this contract.

3.2 PERSONAL PROTECTIVE EQUIPMENT (PPE). The Government shall furnish the contract provider(s) with appropriate PPE. The Government shall be responsible for any repair, cleaning and inventory required for the PPE. This does not include any type of uniform or laboratory coat.

3.3 IDENTIFICATION CREDENTIALS. Wagner IHS will provide each contract provider with the required identification card as applicable.

3.4 FORMS. The Wagner IHS will provide required Government forms used in the performance of services.

3.5 SUPPLIES. Wagner IHS will provide medical and non-medical supplies commonly used in the facility for the care and management of patients and the performance of the requirements of this contract.

3.6 HOUSING. Contractor shall be responsible for all applicable Federal, State, and Local Taxes, Meals, Travel/Transportation, lodging, and per diem.

3.7 SERVICES.

ORIENTATION. The Contractor shall ensure that all new contract providers participate in the Wagner Indian Health Center Facility Orientation with an overlap of services to include one additional day. Providers must arrive on Monday-Friday excluding holiday and participate in the orientation, **which must be completed in the Monday-Friday timeframe**, prior to providing services at the clinic.

3.7.1 Administrative Support. Wagner IHS will provide normal office administrative support services, to include telephone and utilities.

3.7.2 Outpatient Clinic Scheduling. Facility personnel will assist in patient scheduling. Complete administrative control of the patient shall remain with the facility.

3.7.3 Wagner IHS will provide all other ancillary personnel services required, including personnel to move patients or equipment.

3.8 CONTRACTOR FURNISHED ITEMS AND SERVICES

3.8.1 GENERAL. Except for those items or services specifically stated in Section III to be Government furnished, the Contractor shall furnish everything required to perform this contract in accordance with all of its terms.

3.8.2 CONTRACTOR OWNED EQUIPMENT.
The provider shall provide his/her own stethoscope.

3.8.3 INITIAL INSPECTION. Contractor-owned equipment shall be inspected and approved by the biomedical equipment maintenance section prior to use in the facility.

4. GENERAL INFORMATION

4.1 PERSONNEL.

4.1.1 POINT OF CONTACT. The Contractor shall provide a point of contact who shall be responsible for the performance of the work. The point of contact shall have full authority to act for the Contractor on all matters relating to the daily operation of this contract. The Contractor shall designate this individual, in writing, to the Contracting Officer before the contract start date. An alternate may be designated, but the Contract shall identify those times when the alternate shall be the primary point of contact.

4.1.2 CRIMINAL BACKGROUND CHECK REQUIREMENT/CHILD CARE NATIONAL AGENCY CHECK WITH INQUIRIES (CNACI). The Government will conduct at government expense, CNACI and state criminal history checks on individuals providing direct, unsupervised childcare services under this contract. As a minimum, the Contractor shall contact the Wagner IHS background coordinator to obtain all necessary forms and directions for filing background check information in writing and electronically on personnel hired at the beginning of the contract. This action shall take place no later than five (5) working days from contract award date. Contractor shall submit completed personnel background check applications within twenty (20) working days after award date to the background coordinator for processing. Contractor personnel shall be required to be fingerprinted. Requests for personnel hired subsequent to the start contact date shall be submitted to the Government no later than five (5) working days from the employee's first duty day. Contractor shall provide a list of substitute contract personnel fulfilling contingency situations (i.e. absences of certificated contact personnel). Names of substitutes and credentials shall be provided 30 calendar days in advance to accomplish background checks. Contractor personnel receiving unfavorable CNACIs shall not be employed. IHS may employ an individual pending completion of successful background checks.

4.1.2.1 Background checks will be based on fingerprints obtained by a government law enforcement officer and inquiries conducted through the Federal Bureau of Investigation (FBI) and state criminal history repositories.

4.1.2.2 Individuals shall have the right to obtain a copy of any background check pertaining to themselves and to challenge the accuracy and completeness of the information contained in the report.

4.1.2.3 Individuals who have previously received a background check shall provide proof of the check to the Clinical Director or obtain a new one.

4.1.2.4 CONFLICT OF INTEREST. The Contractor shall not employ any person who is an employee of the United States Government if the employment of that person would create a conflict of interest. The Contractor shall not employ any person who is an employee of HHS (Health and Human Services), unless such a person seeks and receives approval in accordance with the Department of HHS.

4.1.2.5 APPEARANCE. Employees are expected to comply with reasonable dress, grooming and hygiene standards based on comfort, productivity, health and type of occupied position. Employee attire will be in good repair, and should not be considered offensive, disruptive or unsafe.

4.1.2.6 Contract providers shall display their identification badge on their outer clothing.

4.1.2.7 PROTECTIVE CLOTHING. When required and supplied by the Government, the provider shall wear special protective clothing and shoe covers. When duties will be performed in specified areas, a disposable protective hood shall be worn to ensure infection control standards are

met. The Government will provide this hood. These items shall remain the property of the Government and shall not be removed from the facility. After use, protective clothing shall be turned in or destroyed as directed by the Infection Control Policy.

4.2 PRIVILEGING REQUIREMENTS. The contract providers' credentials shall be reviewed by the Credentialing Specialist and the Medical Executive Committee. The Contractor is responsible for ensuring that proposed providers possess the requisite credentials to enable granting of privileges by the Wagner Medical Staff sufficient to allow for performance of all tasks identified. Medical Services cannot be performed by a contract Outpatient Mid-level Provider until professional staff membership, and the Great Plains Indian Health Service Governing Body has granted clinical privileges in writing.

4.2.1 Initial applications for clinical privileges, to include credentials action history, malpractice history, and certified true copies of the documents listed below, shall be submitted to the Credentialing Specialist, no later than 3 calendar days after receipt of notice of contract award.

- a. Copy of valid, current unrestricted state license and applicable Board Certification. (Submission of required with proposal).
- b. Documentation of continuing education and training.
- c. Current Basic Life Support (BLS) (Submission required with proposal).
- d. A signed consent for release of information.
- e. Current copy of Drug Enforcement Agency (DEA) Certificate.
- f. A list of all states in which contract provider currently holds or has held a license to practice related services.
- g. A current copy of the proposed contract provider's professional resume.
- h. Certification of current physical examination. The certification shall have a statement signed by the examining physician stating that the employee is free of any contagious diseases. The physical examination shall be current as of 12 months prior to application for privileges.

4.2.2 Applications for privileges will be screened through the National Practitioners Data Bank.

4.2.3 Professional staff appointments and clinical privileges actions will be based on review of documents listed in paragraph 4.2.1 above. Re-appointment shall occur no less than every two years, in accordance with Wagner IHS Medical Bylaws.

4.2.4 DENIAL/TERMINATION OF PRIVILEGES. Actions to limit, suspend, or revoke clinical privileges will be in accordance with the procedures outlined in the Medical Staff Bylaws. The Clinical Director will notify the Contracting Officer, and provide copies of pertinent documentation, as soon as possible when the necessity to exercise such authority becomes apparent. The Contracting Officer will notify the Contractor of the initiation of the revocation process if such action becomes apparent.

4.2.5 REVIEW OF CREDENTIALS PACKAGES. The Government reserves the right to limit the number of credential packages it will evaluate.

4.2 HEALTH REQUIREMENTS

4.3.1 The contract provider shall provide proof of current immunizations (Hep B, MMR, Varicella, Tdap, Flu) status and a current TST within the past year. Current COVID-19 vaccination.

4.3 MEDICAL QUALITY IMPROVEMENT/RISK MANAGEMENT

4.4.1 The Government will evaluate the provider's professional, as differentiated from Administrative, performance under this contract using Quality Improvement standards specified in

paragraphs 2.1 and 2.2. Nothing in this paragraph precludes the Government from also conducting inspections under the Inspection/Acceptance requirement of FAR clause 52.212-4.

4.4 PERFORMANCE EVALUATION

4.5.1 The Wagner IHS clinic will evaluate the contractors' performance under this contract. Evaluations will be provided to the contractor, in writing. These evaluations may be used as a record of contract performance history for the purpose of evaluation in other competitive or non-competitive acquisitions.

4.6 FREEDOM OF INFORMATION ACT (FOIA) PROGRAM. All FOIA requests shall be delivered immediately to the Medical Records Supervisor.

4.7 PHYSICAL ACCESS. The Contractor shall return identification badge to the IT Department upon completion/termination of the contract, as appropriate.

4.7.1 Physical Security. The contractor shall safeguard all government property, including controlled forms provided for contractor use. At the close of each work period, government equipment, facilities, and other valuable materials shall be secured.

4.7.2 Weapons, Firearms, And Ammunition. Contractor employees are prohibited from possessing weapons, firearms, or ammunition, on themselves or within their contractor-owned vehicle or privately-owned vehicle while on the Wagner IHS compound.

4.7.3 Reporting Requirements. Contractor personnel shall report to an appropriate authority any information or circumstances of which they are aware may pose a threat to the security of IHS personnel, contractor personnel or patients.

4.7.4 The contractor shall prohibit their use of keys, issued by the government, by any person other than the contractor's employees and the opening of locked areas by contractor employees to permit entrance of person other than contractor employees engaged in performed of contract work requirement in those areas.

Business Agreement

Pursuant to 45 C.F.R. 164.502(e), the Indian Health Service, as a covered entity, is required to enter into an agreement with a "business associate", as defined in 45 C.F.R. 160.103, under which the business associate must agree to appropriately safeguard Protected Health Information (PHI) that it will use and disclose when performing function, activities or services pursuant to its contract with IHS. Therefore, a business associate agreement shall be enforced to this contract and executed upon award of this contract. See attached Business Associate Agreement.

Invoice Submission:

Invoices shall be submitted through Invoice Processing Platform (IPP).

The IPP website address is: <https://www.ipp.gov>

In compliance with the Office of Management and Budget (OMB) M-15-19 memorandum "Improving Government Efficiency and Saving Taxpayer Dollars Through Electronic Invoicing" directing Federal agencies to adopt electronic invoicing as the primary means to disburse payment to vendors. Invoices submitted under any award will be required to utilize the Invoice Processing Platform (IPP) in accordance with HHSAR 352.232-71, Electronic Submission and Processing of Payment Requests" IPP is a secure, web-based electronic invoicing system provided by the U.S. Department of the Treasury's Bureau of the Fiscal Service, in partnership with the Federal Reserve

Bank of St. Louis (FRSTL).

Contractors are encouraged to register an account with IPP if they have not already done so. If your organization is already registered to use IPP, you will not be required to re-register- however, we encourage you to make sure your organization and designated IPP user accounts are valid and up to date. It will be the responsibility of the contractor to keep active and maintain their SAM.gov account, in order to keep their IPP account active. Payment is sent via E.F.T., through the account registered in SAM.gov.

If you require assistance registering or IPP account access, please contact the IPP Helpdesk at (866) 973-3131 (M-F 8AM to 6PM ET), or IPPCustomerSupport@fiscal.treasury.gov or ihse-invoicingteam@ihhs.gov

Should the contractor feel that use of the IPP would be unduly burdensome, their response to the solicitation should include an explanation of this position, for a determination by the contracting officer to allow non-IPP invoicing.

The contractor shall submit a signed and approved invoice to request payment through IPP, at least bi-weekly. If services are billed at an hourly rate, invoices will include timesheets signed off by the department supervisor.

The Contractor shall include the following information on each invoice in accordance with the Prompt Payment Act (5 CFR Part 1315.9(b):

Contractor's name.

Invoice number and date.

Entire award number/**Contract number** or other authorization number for delivery of goods or services.

Description (including dates of service), price, and quantity of goods and services rendered.

Shipping and payment terms.

UEI number and Taxpayer Identifying Number (TIN); and

Contact name (where practicable), title and telephone number.

Only actual hours worked must be invoiced for, at the Fixed Rate referenced in the award document(s). The government reserves the right to not use all hours enacted under each task order and will de-obligated excess hours at close-out of the task order.

Contractor Authority

The Contractor shall not have the authority to make any commitments, agreements, or representations on behalf of the Government. The Contractor is not authorized to direct Government personnel or to obligate Government funds. Only the Contracting Officer (CO) has the authority to make changes to the terms and conditions of this contract, including changes to the Statement of Work, price, or period of performance. Any direction provided by Government personnel other than the CO shall be considered advisory only and shall not be acted upon unless confirmed in writing by the CO.

Mandatory Background/Security Clearance Requirements: The employees of the Contractor or its subcontractors, are subject to a background investigation and shall comply with Section 231 of the Crime Control Act of 1990, P.L. 101-647; and the Indian Child Protection and Family Violence Prevention Act, P.L. 101-630. In addition, each person shall also comply with the

Privacy Act of 1974 and the HIPPA Act of 1996. Failure to comply with these statutes will result in immediate removal.

Contractor personnel performing work under this contract, which involve regular contact with, or control over, Indian children, are subject to a character investigation in accordance with the "Indian Child Protection and Family Violence Prevention Act" P.L. 101-630. The Indian Health Service Personnel Office will conduct these investigations following award of a contract unless in the judgment of the Contracting Officer, in consultation with the Personnel Officer, investigation may be waived based on prior investigations on file. Until the character investigation has been completed and the Contracting Officer notified, such individual shall not have unsupervised contact with Indian children. This level of security is required and must be maintained for the life of the contract. Before starting work requiring routine access to I.H.S. facilities or systems, each person must complete an FBI National Criminal History Check (Fingerprint Check) adjudicated by an I.H.S. employee using the Office of Personnel Management Personnel Investigations Processing System. Fingerprint cards need to be mailed by USPS, UPS, or Fed Ex, too:

Wagner IHS Healthcare Center
Attn: Fawn Fields
Background Coordinator
111 Washington Ave. NW,
Wagner, South Dakota, 57380

Also, if practical the provider may come to our facility and Mrs. Fields will take the fingerprints.

The only acceptable format for official fingerprints is federal document FD-258 (Rev. 5-15-17) 1110-0046. The fingerprints must be ORIGINAL in ink. The document must be completed in its entirety. The fingerprint form when completed can be sent using USPS, Fed-Ex or any commercial carrier but must be sent to the address and person named above.

In addition, no Contractor or subcontractor employee shall be permitted to perform work under this contract if listed on the HHS Office of Inspector General List of Excluded Individuals/Entities (LEIE), <http://exclusions.oig.hhs.gov>. As soon as practicable prior to the start of work, the performance of the work the Contractor shall provide to the Contracting Officer the names of all individuals to be used in performance of work for screening against the LEIE.

The Contractor is responsible for conducting security pre-clearance investigations in sufficient depth to ensure that each Contractor or subcontractor employee referred to I.H.S. is not on the LEIE and can obtain a favorable fingerprint clearance. Each security preclearance shall be conducted sufficiently in advance of the start of performance to avoid delays caused by denial of access. If this is a Time and Materials, Labor-Hour or Cost Reimbursement contract, the contractor shall not charge for or be reimbursed for labor hours or other costs incurred for employees who are unable to perform due to denial or access, or the excess time required to resolve and clear unfavorable security clearance findings. If this is a Fixed Price contract, denial of access due to security clearance findings shall not be a basis for excusable delay or an increase to the contract amount.

Government will pay the cost to process the contractor's suitability clearances. However, multiple investigations for the same position may, at the Contracting Officers discretion, lead to reduction(s) in the contract price of no more than the cost of the extra investigation(s).

Employees that **DO NOT** have access to LMS, contractors, volunteers, or IHS employees with

IPA's or MOA's to tribal organizations please, click here [IHS Sexual Abuse Prevention website](#). When the contractors have completed and passed the training, please have them send a copy of the last page of the training with Name, Date, and Time to their respective Department Supervisor, COR, and Kimmen LeBeau, e-mail: Kimmen.LeBeau@ihs.gov

Request Background documents from the contractor, to complete the clearance process, should be mailed/FedEx to the designated background coordinator within 5-7 days.

Since the background clearance process may take up to 2 weeks to be processed and cleared, candidates coming on board should have their complete background documents properly filled out, Employee Health Records, and fingerprint cards, sent to the background coordinator 10 days before their scheduled start date. Failure for delay in getting the required background documents into the Background coordinator will be reflect on contractors rating performance and entered into CPARS.

Mandatory Employee Health Records Requirements:

Clearance from the Employee Health Nurse needs to be obtained prior to the contractor arriving at the facility, this means all records have been collected and reviewed by the EHN. The EHN will mark the contractor as "Yes" in the Cleared Column of the database when they have been cleared by the EHN in the Onboarding Database.

The required records are:

- Two (2) documented vaccinations of MMR or a titer showing immunity to Mumps, Measles (Rubeola), Rubella
- Three (3) documented vaccinations of hepatitis b or a titer showing immunity to hepatitis B
- Two (2) documented vaccinations of varicella or a titer showing immunity to varicella
- COVID-19: Completed primary series along with most recent bivalent booster vaccine
 - Medical and Religious Exemptions may be submitted – will need to be reviewed and approved / disapproved by EHC
 - Appeals process not valid for contractors, only Federal Employees
- T-dap within the last 10 years
- PPD (purified protein derivative) or QuantiFERON- TB Gold within the last year or chest x-ray within last five (5) years to rule out Tuberculosis
 - Neither item can expire within contracted time
 - If contractors were to extend and the PPD/Gold/X-ray were to expire during the extension time frame, the expiring items must be completed and submitted to CRHC prior to extension being signed by CRHC
- Flu vaccine for current Flu Season if contracted between Oct 1 and March 31
- Fit Test within the last year to a mask that FTIHS carries.
 - Fit test cannot expire within contracted time
 - If contractors were to extend and the fit test were to expire during the extension time frame, a new fit test must be completed and submitted to FTIHS prior to extension being signed by FTIHS

Contractors are required to follow any Employee Health or Infection Control mandates given by Indian Health Services (IHS) Headquarters, IHS Great Plain Areas, IHS Wagner Indian Health Center. Listed are examples of mandates that would need to be follow as of 1.24.2023, these mandates are fluid and always changing

- Mandatory COVID Bivalent Booster Vaccination